

Recommendations for referral of adult patients with Chronic Kidney Disease (CKD) to UHDB renal services

Chronic Kidney disease (CKD) is defined as abnormalities in kidney function or structure (or both) present for more than 3 months with associated health implications. University Hospitals Derby and Burton Renal Unit provides 24/7 nephrology services to our population in Derbyshire and South Staffordshire. Please see below for our recommendations for referral to our service. Further information on CKD diagnosis, investigation and management is available on Pathfinder.

General Information about referring to Renal Medicine:

In assessing and triaging referrals, the following information is very valuable to enable us to plan outpatient review;

- Historical blood and uACR results showing progression of CKD. This is particularly valuable as we assess patients from different areas and do not always have access to previous blood test results
- Medication history including any recently started medication.
- Blood pressure control, urinary symptoms and urine dipstick results
- Results of renal ultrasound if undertaken

Please take into account patients' wishes and comorbidities when considering the appropriateness of renal referral.

Referral criteria:

- a 5-year risk of needing renal replacement therapy of greater than 5% (measured using the 4-variable Kidney Failure Risk Equation)
- a urine albumin creatinine ratio (uACR) of 70 mg/mmol or more, unless known to be caused by diabetes and already appropriately treated
- an uACR of more than 30 mg/mmol, together with haematuria
- a sustained decrease in eGFR of 25% or more and a change in eGFR category within 12 months
- a sustained decrease in eGFR of 15 ml/min/1.73 m² or more per year
- hypertension that remains poorly controlled (above the person's individual target) despite the use of at least 4 antihypertensive medicines at therapeutic doses
- known or suspected rare or genetic causes of CKD
- suspected renal artery stenosis.

Note:

Stage 5 (eGFR <15): This usually requires immediate referral or discussion

Please ring the registrar on call (9am-10pm 7 days a week) via the UHDB switchboard and we will consider the best way to review the patient promptly

Please consider referral to alternative specialties in the following cases:

- Simple renal cysts: these are common incidental findings in older patient and do not require renal review, we are happy to provide advice and guidance regarding cysts in younger patients
- Complex cysts: please make a referral to urology
- Renal stone disease and recurrent infections: please discuss or refer to urology

Document title	Recommendations for referral of adult patients with Chronic Kidney Disease (CKD) to UHDB renal services		
Revision	3	Document Reference	CHISCP55
Reason for review	Routine review		
Review history	Version/Date	Authors	Reason for review
	V1 Jul 2021	Dr Lis Crowley and Dr Nitin Kolhe	New Document
	V2 Nov 2023	Dr Lisa Crowley, Dr P Blackwell, Mrs H Seddon	Routine review
Approval	Date	Name(s)	Designation
Secondary Care UHDB	14.01.26	Dr Khai Ping Ng, Prof Taal	Consultant Nephrologists
Primary Care	14.01.26	Dr G Strachan	GP Clinical lead for Men's health, Urology and Renal medicine
Laboratory (Derbyshire Pathology)	06.02.26	Mrs J Williams	Consultant Clinical Scientist